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Healthcare Intelligence Briefing

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Insights

HIGH PRIORITY *AIHW · Clinical*

Diabetes is a major driver of hospital activity, accounting for 11% of all hospitalisations (1.3 million) in 2023-24, with 95% recorded as an additional diagnosis, and the number of diabetes-related hospitalisations increased 59% between 2012-13 and 2023-24.

Evidence "Over 1.3 million hospitalisations were associated with diabetes in 2023–24. This represents about 11% of all hospitalisations in Australia. Of the 1.3 million hospitalisations associated with diabetes, 4.9% were recorded as the principal diagnosis (the diagnosis largely responsible for hospitalisation) and around 95% were recorded as an additional diagnosis"
Section: Hospitalisations · Source: Diabetes: Australian facts

Why this priority Diabetes-associated hospitalisations account for 11% of all Australian hospitalisations and rose 59% over a decade, providing strong current clinical, utilisation and population impact. Cost implications and private-hospital-specific exposure are not supplied, but the volume and trend warrant attention now.

HIGH PRIORITY *AIHW · Clinical*

Almost 1 in 5 women (18%) who gave birth in hospital in 2023-24 had gestational diabetes, a rate that has doubled since 2012-13 (from 9.3% to 18.9%), with the highest incidence among women born in Southern and Central Asia (33.1%) and those aged 45-49 (32%).

Evidence "Almost 1 in 5 (18% or 49,800) females aged 15–49 who gave birth in hospital in 2023–24 had been diagnosed with gestational diabetes during their pregnancy, according to the National Hospital Morbidity Database (NHMD). After adjusting for changes in the age structure of the population over time, the incidence of gestational diabetes doubled in Australia between 2012–13 and 2023–24."
Section: Gestational diabetes · Source: Diabetes: Australian facts

Caveat The rising incidence is likely driven by several factors including increasing maternal age, higher rates of maternal overweight and obesity, and a growing proportion of higher risk ethnic groups in the population. The introduction of new diagnostic guidelines across all states and territories between 2011 and 2013 is also likely to have contributed towards the increasing rates.

Why this priority Gestational diabetes now affects about 1 in 5 hospital births and has doubled since 2012-13, a material maternal-health development with strong clinical and change signals. The diagnostic-guideline caveat tempers certainty but does not remove the need for attention; cost and private-payer specifics remain unclear.

MEDIUM PRIORITY *AIHW · Clinical*

In 2023-24, there were 7,000 lower limb amputations and 3,300 weight loss procedures among people with type 2 diabetes, with amputation rates 3.9 times higher in males and obesity surgery rates twice as high in females.

Evidence "In 2023–24, there were 3,300 weight loss procedures and 7,000 lower limb amputations undertaken for people with type 2 diabetes."
Section: Summary - Hospitalisations and procedures · Source: Diabetes: Australian facts

Caveat The number and rate of hospitalisations for procedures reported in this section should be interpreted with caution. Hospital procedures reported using the NHMD may represent many individuals undergoing these procedures, or in some cases, a single individual undergoing multiple procedures over time as they require further follow-up and treatment.

Why this priority The 7,000 lower limb amputations and 3,300 weight loss procedures among people with type 2 diabetes are severe current procedure burdens, with notable sex differences and a plausible private-payer link through obesity surgery. The absence of trend and the caveat that procedures may include repeat episodes limit priority; cost and private-specific data are missing.

MEDIUM PRIORITY *AIHW · Clinical*

In 2023-24, women with pre-existing diabetes had significantly higher rates of caesarean section (70.8% vs 42.0%), preterm birth (26.3% vs 7.4%), and pre-eclampsia (18.4% vs 3.8%) compared to women without diabetes, indicating a high-risk pregnancy population.

Evidence "In 2023–24, compared to mothers without diabetes, mothers living with pre-existing diabetes (and to a lesser extent, gestational diabetes), were more prone to complications during pregnancy and intervention in childbirth. After adjusting for differences in the age structure of the populations, such complications and interventions include caesarean section, induced labour, preterm birth, maternal care for excessive foetal growth and pre-eclampsia (Figure 1)."
Section: Diabetes-related complications - Pregnancy complications · Source: Diabetes: Australian facts

Why this priority Pre-existing diabetes is associated with markedly higher caesarean, preterm birth and pre-eclampsia rates, a high-impact obstetric comparator with strong utilisation implications. The affected group is small and no trend or private-specific data are supplied, so this warrants investigation but not High Priority.

MEDIUM PRIORITY *ABS · Clinical*

Diabetes prevalence has increased from 3.3% in 2001 to 5.3% in 2022, with 1.3 million Australians now living with the condition.

Evidence "Diabetes prevalence has slowly increased from 3.3% in 2001 to 5.3% in 2022."
Section: Diabetes · Source: National Health Survey, 2022

Why this priority Relevant and supported by a sustained 21-year rise in diabetes prevalence and a large affected population of 1.3 million Australians, giving strong trend and population evidence. The change is gradual and utilisation, cost, and direct private-health impacts are not established, supporting Medium rather than High.

WATCHLIST *AIHW · Watchlist*

In 2024, only 30% of people with diabetes attending services for diabetes care met the target HbA1c of ≤7%, with median HbA1c at 7.9% (7.7% for type 1 and 8.0% for type 2), indicating suboptimal glycaemic control in the majority.

Evidence "According to data from the Australian Diabetes Clinical Quality Registry, in 2024, glycaemic control targets were inadequately met, with the median HbA1c for both type 1 diabetes (7.7%) and type 2 diabetes (8.0%) being above target. Similar proportions of people with type 1 diabetes (31%) and type 2 diabetes (29%) met the target HbA1c of ≤7% (or ≤53 mmol/mol)."
Section: Pathology - Page highlights · Source: Diabetes: Australian facts

Caveat Data are from the Australian Diabetes Clinical Quality Registry, which may not be representative of all people with diabetes.

Why this priority Suboptimal glycaemic control, with most people attending diabetes services above the HbA1c target, is a relevant quality-of-care signal. However, the registry population may not be representative, and no trend, utilisation scale, or direct private-health impact is established, so this is worth monitoring rather than treating as an immediate material development.

WATCHLIST *ABS · Watchlist*

One in seven (14.4%) adults have used e-cigarette or vaping devices, while current daily smoking has declined to 10.6% in 2022 from 22.4% in 2001.

Evidence "One in seven (14.4%) adults have used e-cigarette or vaping devices."
Section: Smoking and vaping · Source: National Health Survey, 2022

Why this priority Relevant emerging signal from vaping prevalence (one in seven adults having ever used e-cigarettes) alongside a sustained decline in daily smoking. Current health, utilisation, and cost impacts are not established, so this warrants monitoring rather than medium or high prioritisation.

INFORMATIONAL *AIHW · Clinical*

Diabetes disproportionately affects disadvantaged populations: in 2022-24, prevalence was 3.5 times higher in the lowest socioeconomic areas than the highest, and in 2023-24, hospitalisation rates were 2.3 times higher in Remote and very remote areas than Major cities, with the disparity more pronounced among females for hospitalisations.

Evidence "In 2022–24, the age-standardised prevalence of diabetes among people living in the lowest socioeconomic areas was 3.5 times as high as the prevalence for those living in the highest socioeconomic areas, according to the NHMS data (Figure 4)."
Section: All diabetes - Variation by priority population groups · Source: Diabetes: Australian facts

Why this priority This is a relevant cross-sectional equity finding showing large diabetes prevalence and hospitalisation disparities by socioeconomic area and remoteness, but there is no demonstrated trend or direct private-health connection. It provides useful population-health context rather than a material current development that requires near-term investigation.

INFORMATIONAL *AIHW · Clinical*

Diabetes contributed to 11% of all deaths in 2024 (around 20,800 deaths), with 71% of these having diabetes as an associated rather than underlying cause, and the death rate was 2.4 times higher in the lowest socioeconomic areas than the highest.

Evidence "Diabetes remains one of the 10 leading causes of death in Australia and was the underlying cause of around 6,000 deaths in 2024. However, it contributed to around 20,800 deaths (11% of all deaths) when associated causes are also considered (77 deaths per 100,000 population)."
Section: Deaths · Source: Diabetes: Australian facts

Caveat Deaths registered in 2024 are based on preliminary data and are subject to further revision by the Australian Bureau of Statistics.

Why this priority The mortality burden is clinically large, with diabetes contributing to 11% of all deaths and a notable socioeconomic disparity, but this is a static cross-sectional mortality snapshot with no trend or private-health-specific signal. Preliminary death-registration data also limits certainty, so the insight is contextual rather than an active priority.

INFORMATIONAL *AIHW · Clinical*

First Nations adults are 3.2 times as likely to have diabetes as non-Indigenous adults, with a diabetes death rate 4.0 times higher and an ED presentation rate 5.6 times higher, indicating a severe health inequity.

Evidence "First Nations adults were 3.2 times as likely as non–Indigenous adults to have diabetes, after adjusting for age (ABS 2025a; ABS 2025b)."
Section: Summary - Variation by priority population groups · Source: Diabetes: Australian facts

Why this priority The large First Nations disparities in diabetes prevalence, mortality and ED presentations are clinically important, but the evidence is cross-sectional and lacks direct private-health or Medibank-relevant exposure. This is public-health equity context rather than a current development warranting near-term action.

INFORMATIONAL *ABS · Clinical*

Three-quarters (74.5%) of adults with high measured blood pressure did not report having hypertension, indicating a large gap between measured and self-reported diagnosis.

Evidence "Three in four (74.5%) adults with high measured blood pressure did not report having hypertension."
Section: Hypertension and high measured blood pressure · Source: National Health Survey, 2022

Why this priority Relevant as a major clinical gap in hypertension awareness, with a large discrepancy between measured and self-reported hypertension. However, this is a cross-sectional snapshot with no demonstrated trend, utilisation, cost, or direct private-health impact, so it is best treated as informational.

INFORMATIONAL *ABS · Pharmaceutical*

83.0% of people with one or more chronic conditions were dispensed at least one PBS medication in the six months before or after their interview, compared to 68.6% of the total population.

Evidence "Over eight in ten (83.0%) of people with one or more chronic conditions were dispensed at least one PBS medication."
Section: Pharmaceutical Benefits Scheme (PBS) medications · Source: National Health Survey, 2022

Why this priority Relevant pharmaceutical utilisation signal showing high PBS dispensing among people with chronic conditions. The evidence is a single cross-sectional comparison without trend, cost, or private-insurance impact, so it is informational.

INFORMATIONAL *ABS · Pharmaceutical*

People with chronic conditions are significantly more likely to use over-the-counter pain relief: 52.6% used paracetamol and 25.3% used ibuprofen, compared to 34.5% and 16.1% respectively for those without a chronic condition.

Evidence "People with one or more chronic conditions were more likely to have used paracetamol than people with no chronic condition (52.6% compared to 34.5%)"
Section: Over-the-counter (OTC) medications · Source: National Health Survey, 2022

Why this priority Relevant self-care and OTC medication use difference among people with chronic conditions, but this is a static cross-sectional comparison with no trend, cost, or private-health impact demonstrated. Informational only.

INFORMATIONAL *ABS · Clinical*

More than one in four (26.8%) adults exceeded the Australian Adult Alcohol Guideline in 2022, with males (35.8%) more likely than females (18.1%) to exceed it, and over a third (36.1%) of young adults aged 18-24 doing so.

Evidence "More than one in four (26.8%) adults exceeded the Australian Adult Alcohol Guideline in 2022"
Section: Alcohol consumption · Source: National Health Survey, 2022

Why this priority Relevant alcohol risk-factor prevalence affecting a large proportion of adults, but this is a cross-sectional snapshot without trend, cost, utilisation, or direct private-health impact. Informational.

INFORMATIONAL *ABS · Clinical*

Almost two-thirds (65.8%) of adults were overweight or obese, and more than two-thirds (67.9%) had a measured waist circumference putting them at increased risk of disease.

Evidence "More than two in three (67.9%) adults had measured waist circumference that put them at an increased risk of disease in 2022"
Section: Waist circumference and BMI · Source: National Health Survey, 2022

Why this priority Relevant obesity and waist-circumference risk profile affecting a large share of adults, but the evidence is a static cross-sectional finding with no demonstrated change, utilisation, cost, or private-health effect. Informational.

INFORMATIONAL *ABS · Clinical*

People in Tasmania were more likely to have at least one chronic condition (58.4%) than any other state or territory.

Evidence "People in Tasmania were more likely to have at least one chronic condition (58.4%) than any other state or territory"
Section: State and territory findings · Source: National Health Survey, 2022

Why this priority Relevant state-level chronic condition prevalence comparison, but this is a static cross-sectional finding with no trend, magnitude, cost, or private-health impact. Informational.

INFORMATIONAL *ABS · Clinical*

Adults in the ACT were more likely to have never smoked (67.6%) and less likely to be current daily smokers (5.9%) than any other state or territory.

Evidence "Adults in the ACT were more likely to have never smoked (67.6%) and less likely to be current daily smokers (5.9%) than any other state or territory"
Section: State and territory findings · Source: National Health Survey, 2022

Why this priority Relevant state-level smoking profile difference for the ACT, but no trend, magnitude, cost, or private-health impact is demonstrated. Informational.

INFORMATIONAL *ABS · Clinical*

More people aged 15 years and over in Victoria met the physical activity guidelines in 2022 than in 2017-18 (16.8% compared to 24.5%).

Evidence "More people aged 15 years and over in Victoria met the physical activity guidelines in 2022 than in 2017–18 (16.8% compared to 24.5%)"
Section: State and territory findings · Source: National Health Survey, 2022

Why this priority Relevant physical activity trend evidence, but the supplied finding is internally inconsistent: it claims more people met the guidelines in 2022 while reporting 16.8% versus 24.5% in 2017-18, making the direction and magnitude unreliable. Informational.